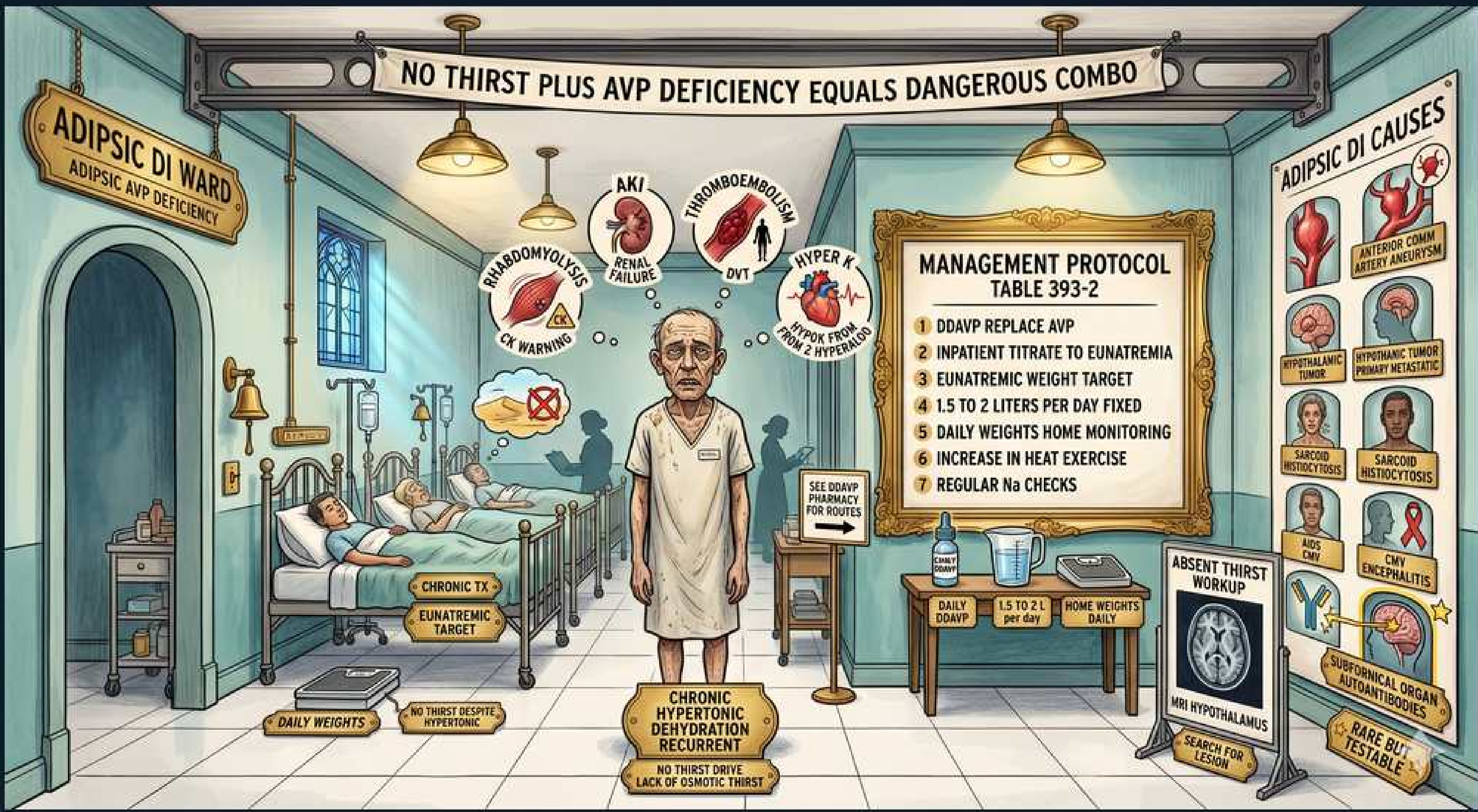
Adipsic diabetes insipidus — absent thirst plus AVP deficiency



1. Adipsic DI — dangerous combo

WHAT YOU SEE

Banner: no thirst plus AVP deficiency equals dangerous combo

WHAT IT ENCODES

Adipsic DI combines AVP deficiency with loss of the thirst mechanism, removing the body's last defense against hypernatremia. Patients cannot sense the need to drink and develop chronic hypertonic dehydration. It is among the most dangerous forms of DI.

BOARD TRAP

Ordinary central DI is protected by thirst; adipsic DI removes that safeguard, so hypernatremia recurs despite DDAVP.

2. Absent thirst / osmoreceptor lesion

WHAT YOU SEE

Adipsic AVP deficiency ward label, absent thirst

WHAT IT ENCODES

The defect is hypothalamic osmoreceptor damage abolishing both AVP release and thirst. Common causes include craniopharyngioma, anterior communicating artery aneurysm clipping, and hypothalamic surgery. Patients drink by schedule, not sensation.

BOARD TRAP

Adipsia localizes to the anterior hypothalamic osmoreceptor/thirst center — distinct from the AVP-storage posterior pituitary.

3. Chronic hypertonic dehydration

WHAT YOU SEE

Chronic hypertonic dehydration recurrent, no thirst despite hypertonic

WHAT IT ENCODES

Without thirst, patients chronically run high sodium and osmolality. Management relies on fixed daily fluid prescriptions and weight monitoring rather than symptoms. Recurrent hypernatremia is the norm.

BOARD TRAP

Do not rely on the patient feeling thirsty — adipsic DI needs scheduled fixed fluid intake and daily weights, not symptom-driven drinking.

4. DDAVP replacement

WHAT YOU SEE

Management protocol step 1: DDAVP replace AVP

WHAT IT ENCODES

DDAVP is given to replace deficient AVP and reduce water losses. Because thirst is absent, dosing must be paired with a fixed water intake. Over-replacement plus excess intake causes hyponatremia.

BOARD TRAP

In adipsic DI both DDAVP and water are prescribed in fixed amounts; mismatch swings the patient between hyper- and hyponatremia.

5. Inpatient titrate to eunatremia

WHAT YOU SEE

Protocol step 2: inpatient titrate to eunatremia

WHAT IT ENCODES

Initial titration is done inpatient to establish a stable DDAVP dose and daily fluid volume targeting normal sodium. A reproducible regimen is then continued at home. Frequent labs guide adjustment.

BOARD TRAP

Outpatient guesswork is unsafe — adipsic DI regimens are calibrated inpatient against measured sodium before discharge.

6. Euvolemic weight target

WHAT YOU SEE

Protocol step 3: euvolemic weight target

WHAT IT ENCODES

A target euvolemic body weight is set as a daily anchor; weight gain signals water retention and weight loss signals dehydration. Daily home weights drive fluid adjustments. Weight is a proxy for water balance.

BOARD TRAP

Daily weight, not thirst, is the home monitoring tool — a rising weight means cut fluids, falling weight means add fluids.

7. Fixed 1.5-2 L/day

WHAT YOU SEE

Protocol step 4: 1.5 to 2 liters per day fixed

WHAT IT ENCODES

A fixed daily fluid prescription (about 1.5-2 L) replaces absent thirst-driven intake. The amount is adjusted for climate and activity. Consistency prevents large osmolality swings.

BOARD TRAP

Fluid is prescribed by volume, not by thirst; fixed intake must increase with heat and exercise to avoid hypernatremia.

8. Daily weights monitoring

WHAT YOU SEE

Protocol step 5: daily weights home monitoring

WHAT IT ENCODES

Patients track daily weights at home as the primary feedback signal for fluid titration. Combined with periodic sodium checks this prevents extremes. Education and adherence are critical.

BOARD TRAP

Missed daily weights are the usual reason adipsic DI patients present with severe hypernatremia or hyponatremia.

9. Increase fluid in heat/exercise

WHAT YOU SEE

Protocol step 6: increase in heat exercise

WHAT IT ENCODES

Because thirst cannot signal increased need, fluid intake must be proactively raised during heat exposure or exercise. Insensible losses rise without compensatory drinking. This prevents acute hypernatremic crises.

BOARD TRAP

Hot weather or exertion is a classic trigger for adipsic DI hypernatremia — pre-emptively increase the fixed fluid order.

10. Regular Na checks

WHAT YOU SEE

Protocol step 7: regular Na checks

WHAT IT ENCODES

Periodic serum sodium measurements verify the regimen and catch drift early. They guide DDAVP and fluid adjustments alongside daily weights. Frequency is individualized to stability.

BOARD TRAP

Even with stable weights, intermittent sodium checks are needed because adipsic patients give no symptomatic warning.

11. AKI / rhabdomyolysis risk

WHAT YOU SEE

Bubble: rhabdomyolysis renal failure, AKI warning

WHAT IT ENCODES

Severe hypertonic dehydration can precipitate AKI and rhabdomyolysis from volume depletion and cellular injury. These are recognized complications of recurrent hypernatremia. Volume restoration is protective.

BOARD TRAP

Hypernatremic dehydration in adipsic DI can cause rhabdomyolysis and AKI — monitor CK and creatinine during crises.

12. Thromboembolism / VTE risk

WHAT YOU SEE

Bubble: thromboembolism DVT, hyperviscosity

WHAT IT ENCODES

Hypertonic dehydration raises blood viscosity and predisposes to venous thromboembolism. This adds a prothrombotic complication to the dehydration risk. Adequate hydration mitigates it.

BOARD TRAP

Dehydration-induced hyperviscosity in adipsic DI increases VTE risk — another reason to maintain the fixed fluid intake.

13. Adipsic DI causes panel

WHAT YOU SEE

Right wall adipsic DI causes: craniopharyngioma, ACoA aneurysm, sarcoid, trauma

WHAT IT ENCODES

Causes mirror osmoreceptor-region damage: craniopharyngioma, anterior communicating artery aneurysm or its clipping, hypothalamic surgery/trauma, and granulomatous disease like sarcoid. All injure the anterior hypothalamic thirst center. Etiology guides prognosis.

BOARD TRAP

ACoA aneurysm surgery and craniopharyngioma are classic adipsic DI causes because they damage the anterior hypothalamic thirst osmoreceptors.

14. Absent thirst workup / MRI

WHAT YOU SEE

Absent thirst workup, MRI, no hypernatremic thirst

WHAT IT ENCODES

Diagnosis is confirmed by demonstrating no thirst despite induced hypernatremia, plus MRI of the hypothalamus. The lack of a thirst response during hypertonic challenge is the defining test. Imaging identifies the structural lesion.

BOARD TRAP

Adipsia is proven by absent thirst during a supervised hypertonic challenge — not by history alone, since patients underreport symptoms.